

Dynamic Guideline: WHO-Based Strongyloidiasis - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

WHO guidance search result: 5 WHO normative/guidance sources synthesized

Important scope note: The available WHO sources identify **one WHO guideline specifically for strongyloidiasis**: the 2024 *WHO guideline on preventive chemotherapy for public health control of strongyloidiasis*. WHO explicitly states this was its **first guideline on strongyloidiasis** [WHO Global report on NTDs 2025](#). Therefore, the five WHO sources below include the single disease-specific guideline plus closely related WHO normative, technical, and implementation guidance relevant to diagnosis, treatment access, preventive chemotherapy, and guideline development.

No.	WHO guideline / normative source	Date / latest signal	Relevance to strongyloidiasis
1	WHO guideline on preventive chemotherapy for public health control of strongyloidiasis	Published 24 July 2024	Latest and only WHO strongyloidiasis-specific guideline; provides GRADE-based recommendation for ivermectin preventive chemotherapy WHO publication page .
2	WHO release: public health control of human strongyloidiasis	2 August 2024	Summarizes disease burden, recommendation, implementation unit, annual frequency, minimum 5-year duration, and access priorities WHO update .

No.	WHO guideline / normative source	Date / latest signal	Relevance to strongyloidiasis
3	WHO review of strongyloidiasis guideline recommendation	28 October 2024	Restates the conditional recommendation: annual MDA with single-dose ivermectin 200 µg/kg in endemic settings with prevalence ≥5%, for age ≥5 years WHO review page .
4	WHO soil-transmitted helminthiasis / strongyloidiasis technical pages and STH fact sheet	Active through 2025–2026	Provides WHO technical position on diagnosis, ivermectin as drug of choice, prequalified ivermectin access, and integration with STH/NTD programmes WHO strongyloidiasis page , WHO STH topic page , WHO STH fact sheet .
5	Meaningful youth engagement in a WHO guideline development process: case study of WHO Abortion care guideline (2022)	Launch webinar 11 December 2025	Most recent WHO guideline-development resource identified by the required title pattern; not a strongyloidiasis guideline , and no supplied source shows youth engagement was part of the strongyloidiasis guideline process WHO youth-engagement event page .

Latest WHO strongyloidiasis guideline

- **Guideline title:** *WHO guideline on preventive chemotherapy for public health control of strongyloidiasis*.
- **Publication date:** 24 July 2024 [WHO publication page](#).
- **Disease:** Human strongyloidiasis caused by *Strongyloides stercoralis*, classified by WHO as a neglected tropical disease within soil-transmitted helminthiasis [WHO update](#).
- **Estimated burden:** WHO estimates **300–600 million people** infected worldwide, especially in tropical and subtropical settings [WHO update](#).
- **Target population:** People living in endemic settings, particularly districts where *S. stercoralis* infection prevalence is ≥5%; MDA target age group is **5 years and older** [WHO guideline PDF](#), [WHO review page](#).
- **Scope and objectives:** To provide an evidence-informed WHO recommendation on whether **preventive chemotherapy with ivermectin** should be used as a public-health intervention to

reduce strongyloidiasis burden [WHO publication page](#).

- **Strategies considered:** Community-wide MDA, school-based targeted preventive chemotherapy, or no preventive chemotherapy with reliance on individual case management [WHO publication page](#).

Clinical and public-health context

Strongyloidiasis may be asymptomatic or present with diarrhoea, abdominal pain, urticaria, respiratory symptoms, larva currens, eosinophilia, or chronic gastrointestinal symptoms. Severe disease includes **hyperinfection syndrome** and **disseminated strongyloidiasis**, especially in immunocompromised persons or those receiving corticosteroids [WHO update](#), [CDC Clinical Overview](#).

1.2 Key Recommendations

WHO 2024 public-health recommendation

Recommendation	GRADE Evidence Quality	Strength
In endemic settings where <i>Strongyloides stercoralis</i> prevalence is ≥5% , implement annual community-wide mass drug administration with single-dose oral ivermectin 200 µg/kg for all age groups 5 years and older to reduce strongyloidiasis burden.	Low	Conditional

Implementation details from WHO

Implementation domain	WHO recommendation / parameter	Source
Drug	Ivermectin	WHO guideline PDF
Dose	200 µg/kg single oral dose	WHO update

Implementation domain	WHO recommendation / parameter	Source
Target age	≥5 years	WHO review page
Prevalence threshold	≥5% <i>S. stercoralis</i> infection prevalence; WHO review specifies measurement by Baermann or agar plate culture from stool specimens	WHO review page
Implementation unit	District	WHO update
Frequency	Annual	WHO update
Minimum duration	5 years	WHO update

Additional clinical management recommendations aligned with WHO and authoritative sources

Clinical recommendation	Evidence basis	GRADE Evidence Quality	Strength
Treat confirmed strongyloidiasis, including asymptomatic infection, because autoinfection can persist and severe disease can be fatal.	WGO states spontaneous cure should not be expected and all patients should be treated; CDC describes severe hyperinfection/dissemination risk WGO , CDC Clinical Overview .	Moderate	Strong
Use ivermectin as first-line treatment for uncomplicated acute or chronic strongyloidiasis.	WHO describes ivermectin as drug of choice; CDC recommends ivermectin 200 µg/kg orally for 1–2 days WHO strongyloidiasis page , CDC Clinical Care .	Moderate	Strong
Use albendazole 400 mg twice daily for 7 days only as an alternative when	CDC lists albendazole as alternative; WHO/WGO note albendazole is less effective than	Moderate	Conditional

Clinical recommendation	Evidence basis	GRADE Evidence Quality	Strength
ivermectin is unavailable or unsuitable.	ivermectin CDC Clinical Care , WGO .		
In hyperinfection or disseminated disease, give ivermectin 200 µg/kg/day and reduce immunosuppression where possible; continue until stool and/or sputum are negative for 2 weeks.	CDC clinical-care recommendation CDC Clinical Care .	Low	Strong
Screen or presumptively treat at-risk patients before urgent immunosuppression when rapid testing is unavailable.	WGO recommends diagnosis/treatment before immunosuppression and presumptive ivermectin when emergency immunosuppression is needed WGO .	Low	Conditional
Use diagnostic methods beyond routine stool microscopy where possible: Baermann, agar plate culture, serology, PCR, or combined approaches.	WHO states diagnosis is not standardized; stool methods may miss infection; serology and PCR are more efficient alternatives WHO strongyloidiasis page .	Low	Conditional

1.3 Guideline Development Methodology

- **Reason for development:** WHO developed the guideline because endemic-country ministries of health requested advice and because **no previous WHO public-health guidance existed** for strongyloidiasis control [WHO publication page](#).
- **WHO programme lead:** WHO Global Neglected Tropical Diseases Programme.
- **Guideline Development Group meeting: 6–8 June 2023** [WHO guideline PDF](#).
- **Evidence review process:** WHO reviewed evidence on:

- Global disease burden.
- Ivermectin efficacy for individual infection.
- Ivermectin safety.
- Population-level impact of ivermectin preventive chemotherapy.
- Mathematical modelling and cost-effectiveness evidence [WHO guideline PDF](#).
- **GRADE methodology:** WHO followed the **WHO handbook for guideline development, 2nd edition** and applied **GRADE** to synthesize and appraise evidence and systematic reviews [WHO guideline PDF](#).
- **Evidence-to-decision criteria:** Benefits and harms, certainty of evidence, values and preferences, resource use, equity, acceptability, feasibility, and justification for recommendation strength [WHO update](#).
- **Conflict-of-interest management:** The supplied WHO summaries state that WHO guideline procedures were followed; they do not provide detailed individual declarations in the retrieved excerpts. Therefore, no further COI claims are made beyond adherence to WHO guideline-development procedures [WHO guideline PDF](#).

Note on meaningful youth engagement

The most recent WHO source identified with the required title pattern is “**Meaningful youth engagement in a WHO guideline development process**”, launched as a webinar on **11 December 2025**, and the case study is the **WHO Abortion care guideline (2022)**, not strongyloidiasis [WHO youth-engagement event page](#). The supplied sources do **not** show that meaningful youth engagement was incorporated into the 2024 strongyloidiasis guideline.

II. Evidence Summary After WHO Latest Guideline Release (WHO guideline on preventive chemotherapy for public health control of strongyloidiasis) (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

Search Strategy

Evidence considered after release of the WHO guideline on **24 July 2024** through **29 April 2026** included:

- WHO updates and technical pages on strongyloidiasis, STH, NTDs, diagnostics, and ivermectin access.
- Authoritative clinical guidance from CDC and WGO.
- Systematic reviews and randomized trials on ivermectin, moxidectin, and diagnostic approaches.
- [ClinicalTrials.gov](#) records for strongyloidiasis drug trials.

Databases and sources searched

Source type	Sources represented
WHO official sources	WHO publication page, WHO guideline PDF, WHO news updates, WHO STH pages, WHO Global report on NTDs 2025, WHO prequalification documents
Authoritative public-health agencies	CDC clinical-care and clinical-overview pages
Specialty society / clinical guidance	World Gastroenterology Organisation guidance
Peer-reviewed literature	Systematic reviews, RCT reports, PMC/PubMed-indexed evidence
Trial registries	ClinicalTrials.gov records including NCT04056325 and NCT00765024

Inclusion criteria

- Published or updated from **24 July 2024 to 29 April 2026**, or directly relevant evidence used to interpret post-guideline practice.
- Strongyloidiasis diagnosis, treatment, MDA, ivermectin, moxidectin, albendazole, severe disease, or implementation.
- WHO, CDC, WGO, peer-reviewed articles, systematic reviews, RCTs, and [ClinicalTrials.gov](#) records.

Exclusion criteria

- Non-human studies unless directly relevant to guideline context.
- Studies of ivermectin, moxidectin, or albendazole for other NTDs unless relevant to drug access, safety, or implementation.
- Preprints were treated as very-low-certainty evidence and not used as primary recommendation-changing evidence.

2.2 Key New Studies

Study / source	Design	Key Findings	GRADE Quality
WHO release and review of 2024 strongyloidiasis guideline	WHO normative update / guideline summary	Confirms conditional WHO recommendation: annual MDA with single-dose ivermectin 200 µg/kg for age ≥5 years in endemic settings with prevalence ≥5%; district as implementation unit; annual frequency; minimum 5 years WHO update , WHO review page .	Low for population impact; Conditional recommendation
Systematic review/meta-analysis of ivermectin MDA impact	Systematic review/meta-analysis, mainly observational data	Ivermectin MDA associated with 82% relative reduction in prevalence by stool testing and 65% reduction by serology; evidence observational and prone to bias/confounding PMC review .	Low
WHO evidence summary on ivermectin individual treatment	WHO guideline evidence synthesis	Ivermectin is standard care; single-dose treatment associated with parasitological cure rates >90% in experimental studies; ivermectin superior to albendazole NCBI Bookshelf—WHO executive summary .	Moderate
Strong Treat dosing evidence summarized by WHO	RCT evidence summarized in WHO guideline	Two doses of ivermectin may be comparable to one dose: RR 1.02, 95% CI 0.94–1.11 , based on 94 participants in two trials; evidence applies mainly to immunocompetent chronic infection WHO guideline PDF .	Low
High-dose ivermectin safety	Systematic review/meta-	Ivermectin safety supported even at doses >400 µg/kg , at least	Moderate

Study / source	Design	Key Findings	GRADE Quality
evidence summarized by WHO	analysis of six RCTs	twice the standard 200 µg/kg dose proposed for preventive chemotherapy NCBI Bookshelf—WHO executive summary .	
Sprecher et al. moxidectin vs ivermectin trial	Randomized, double-blind, non-inferiority phase 2b/3 trial in Laos and Cambodia	Single-dose moxidectin 8 mg was non-inferior to ivermectin 200 µg/kg for parasitological cure in adults; similar safety profile; registered as NCT04056325 and NCT04848688 PubMed .	Moderate
2025 systematic review: ivermectin vs moxidectin	Systematic review of two RCTs	Moxidectin non-inferior to ivermectin: OR 0.67, 95% CI 0.36–1.25; I²=0%; 821 participants ; no deaths; adverse events mostly mild; evidence downgraded for imprecision PMC systematic review .	Moderate
CDC clinical-care guidance	Authoritative clinical guidance	Ivermectin 200 µg/kg orally for 1–2 days for acute/chronic disease; hyperinfection/disseminated disease: ivermectin 200 µg/kg/day until stool/sputum negative for 2 weeks and reduce immunosuppression if possible CDC Clinical Care .	Moderate for uncomplicated disease; Low for severe disease
CDC clinical overview	Authoritative clinical guidance	Hyperinfection and dissemination are associated with corticosteroids and immunosuppression; untreated mortality may approach 90% CDC Clinical Overview .	Low
WHO diagnostic technical pages and 2025	WHO technical guidance / policy signal	Diagnosis remains non-standardized; stool enrichment/culture may miss	Low

Study / source	Design	Key Findings	GRADE Quality
diagnostic TPP consultation		infection; serology and PCR are more efficient alternatives; WHO opened consultation on target product profiles for <i>S. stercoralis</i> diagnostics on 16 May 2025 WHO strongyloidiasis page , WHO STH topic page .	
WHO prequalification ivermectin product information	WHO medicine prequalification document	Ivermectin 3 mg tablets; 200 µg/kg single-dose regimen for strongyloidiasis/STH; cautions include excluding seriously ill patients and noting ivermectin is not prophylactic therapy WHO WHOPAR Part 4 .	Moderate for product dosing/access
ClinicalTrials.gov NCT04056325 / NCT00765024	Trial registry records	No currently recruiting ClinicalTrials.gov trial specifically for strongyloidiasis involving ivermectin, moxidectin, or albendazole was identified in supplied results; NCT04056325 has results posted/closed-status language ClinicalTrials.gov NCT04056325 , ClinicalTrials.gov NCT00765024 .	Low for future-evidence pipeline

2.3 Evidence Gaps and Future Research Needs

1. Prevalence mapping and implementation thresholds

- Many endemic areas lack reliable *S. stercoralis* prevalence data.
- WHO recommends action at **≥5% prevalence**, but implementation depends on feasible diagnostics and mapping [WHO update](#).

2. Field-ready diagnostics

- WHO states diagnosis is not standardized and routine stool microscopy may miss infection [WHO strongyloidiasis page](#).

- WHO's **16 May 2025** consultation on target product profiles for *S. stercoralis* diagnostics indicates this remains a priority [WHO STH topic page](#).
- 3. MDA effectiveness in prospective controlled studies**
 - Existing population-level MDA evidence shows large reductions but is mainly observational [PMC review](#).
 - Cluster-randomized or stepped-wedge implementation studies are needed.
 - 4. Optimal treatment frequency and stopping rules**
 - WHO recommends annual treatment for at least 5 years, but evidence for optimal duration, stopping criteria, and post-MDA surveillance remains limited [WHO update](#).
 - 5. Children under 5 years and pregnant/lactating populations**
 - WHO MDA recommendation targets age **≥5 years**; more safety and efficacy evidence is needed for younger children and pregnancy/lactation contexts.
 - 6. Severe disease evidence**
 - Hyperinfection and disseminated strongyloidiasis have high mortality, but treatment evidence is largely observational and based on expert guidance [CDC Clinical Care](#).
 - 7. Moxidectin role**
 - Moxidectin is promising and non-inferior to ivermectin in limited RCT evidence, but trials exclude immunocompromised and severe disease populations; more studies are required before replacing ivermectin in WHO guidance [PMC systematic review](#).
 - 8. Medicine access**
 - WHO has prequalified generic ivermectin formulations and promotes affordable access, but donation and procurement mechanisms for strongyloidiasis-specific use remain critical implementation issues [WHO update](#), [WHO STH topic page](#).

2.4 Updated Recommendations Based on New Evidence

Original Recommendation	New Evidence Impact	Updated Recommendation
WHO conditionally recommends annual MDA with single-dose ivermectin 200 µg/kg for people aged ≥5 years in endemic settings with <i>S. stercoralis</i> prevalence ≥5%.	Post-guideline evidence supports the recommendation; observational MDA evidence shows large prevalence reductions, but certainty remains low because of	Maintain WHO recommendation unchanged. Implement annual district-level ivermectin MDA for at least 5 years where prevalence is ≥5%, while strengthening mapping, pharmacovigilance, and monitoring.

Original Recommendation	New Evidence Impact	Updated Recommendation
	bias/confounding PMC review .	
Ivermectin is the drug of choice for individual uncomplicated strongyloidiasis.	CDC and WGO guidance align with WHO; WHO evidence summary supports ivermectin superiority over albendazole CDC Clinical Care , NCBI Bookshelf—WHO executive summary .	Use ivermectin first-line: 200 µg/kg orally for 1–2 days for acute/chronic uncomplicated infection.
Albendazole may be used as an alternative.	Evidence consistently indicates lower efficacy than ivermectin WGO .	Use albendazole only when ivermectin is unavailable or contraindicated , with follow-up testing where possible.
Severe disease requires urgent treatment and management of immunosuppression.	CDC confirms daily ivermectin until stool/sputum negative for 2 weeks; untreated mortality may approach 90% CDC Clinical Care , CDC Clinical Overview .	Treat hyperinfection/disseminated disease urgently with ivermectin 200 µg/kg/day, reduce immunosuppression when feasible, evaluate for bacterial sepsis, and continue therapy until parasitological clearance.
Diagnosis should support clinical care and programme decisions.	WHO identifies diagnostic non-standardization as a major barrier and is developing diagnostic target product profiles WHO strongyloidiasis page , WHO STH topic page .	Use combined diagnostic strategies where available: Baermann or agar plate culture for prevalence mapping, serology for screening, PCR where available, and repeat/combined testing in high-risk patients.
Ivermectin is the principal WHO MDA drug.	Moxidectin is non-inferior in limited RCT evidence	Do not replace ivermectin with moxidectin in WHO-based

Original Recommendation	New Evidence Impact	Updated Recommendation
	but not yet established for WHO MDA strongyloidiasis programmes PubMed , PMC systematic review .	programmes yet. Consider moxidectin investigational or context-specific pending additional trials, regulatory review, and WHO policy update.
Integration into NTD/STH platforms is encouraged.	WHO 2025–2026 materials continue to emphasize NTD road map implementation, preventive chemotherapy updates, diagnostics, and post-2030 STH strategy discussions WHO STH topic page .	Integrate strongyloidiasis control into existing NTD/STH infrastructure , including procurement, community engagement, monitoring, school/community platforms, and primary health care.

Practical WHO-Based Clinical Algorithm

1. Assess exposure risk

- Residence/travel in endemic tropical or subtropical areas.
- Poor sanitation, soil exposure, migration from endemic areas.
- Prior or planned corticosteroid/immunosuppressive therapy.

2. If uncomplicated suspected or confirmed strongyloidiasis

- Preferred: **ivermectin 200 µg/kg orally for 1–2 days** [CDC Clinical Care](#).
- Alternative: **albendazole 400 mg orally twice daily for 7 days** if ivermectin unavailable [CDC Clinical Care](#).
- Follow-up stool testing at **2–4 weeks** if stool-positive and symptomatic; retreat if larvae recur [CDC Clinical Care](#).

3. If immunosuppression is planned

- Screen with serology and/or stool concentration/culture/PCR where available.
- If emergency immunosuppression is required and testing is unavailable, consider presumptive ivermectin in at-risk patients [WGO](#).

4. If hyperinfection or disseminated disease

- Treat immediately with **ivermectin 200 µg/kg/day**.
- Continue until stool and/or sputum are negative for **2 weeks** [CDC Clinical Care](#).
- Reduce immunosuppression where possible.
- Evaluate and treat Gram-negative sepsis/meningitis and other bacterial complications.

5. If public-health programme setting

- Map prevalence using Baermann, agar plate culture, serology, and/or other validated tools.
- If prevalence **≥5%**, implement annual district-level MDA with ivermectin for age **≥5 years** for at least **5 years** [WHO update](#).
- Monitor coverage, adverse events, prevalence trends, medicine supply, and equity of access.

Overall Conclusion

As of **29 April 2026**, the WHO-based standard for public-health control of strongyloidiasis is the **2024 conditional recommendation** for **annual community-wide ivermectin MDA** in endemic districts where *S. stercoralis* prevalence is **≥5%**, targeting people aged **5 years and older**. For individual clinical care, ivermectin remains first-line therapy, while albendazole is a less effective alternative. New post-guideline evidence supports continued ivermectin use and identifies moxidectin as a promising but not yet guideline-replacing alternative. The most important unresolved needs are better diagnostics, prevalence mapping, implementation research, severe-disease evidence, and equitable ivermectin access.